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Anat Path Reports

* Final Report *

Document Type: Anat Path Reports
Document Date:
Document Status: Auth (Verified)
Document Title: Histology
Encounter info:

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APRPT (Verified)

Patient Name:
MRN:
Location:
Client:
Submitting Phys:

DOB:
Gender: F

Acc #:
Collected:
Received:
Reported:

Final Surgical Pathology Report

Final Pathologic Diagnosis

A. LYMPH NODES, #7; EXCISION:
- FOUR LYMPH NODES, NEGATIVE FOR METASTATIC CARCINOMA (0/4).

B. LYMPH NODES, #11R; EXCISION:
- FOUR LYMPH NODES, NEGATIVE FOR METASTATIC CARCINOMA (0/4).

C. LUNG, RIGHT LOWER LOBE; LOBECTOMY, C1FS:
- ADENOCARCINOMA, MIXED SUBTYPE, WITH PREDOMINANTLY PAPILLARY AND MICROPAPILLARY PATTERNS OF GROWTH.
- THE TUMOR IS WELL-DIFFERENTIATED AND MEASURES 4.0 X 4.0 X 2.0 CM.
- ANGIOLYMPHATIC INVASION IDENTIFIED.
- BRONCHIAL AND VASCULAR MARGINS ARE NEGATIVE.
- VISCERAL PLEURAL INVASION IS IDENTIFIED.
- ONE LYMPH NODE NEGATIVE FOR METASTATIC CARCINOMA (0/1).
- SEE SYNOPTIC REPORT.

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D. LUNG, POSTERIOR SEGMENT RIGHT UPPER LOBE; WEDGE:
- BENIGN LUNG WITH APICAL CAP LIKE FIBROSIS AND SUBPLEURAL CYSTS.

E. LYMPH NODE, 4R; EXCISION:
- FOURTEEN LYMPH NODES, NEGATIVE FOR CARCINOMA (0/14).

F. LUNG, INFERIOR PORTION OF RIGHT MIDDLE LOB; WEDGE:
- BENIGN LUNG PARENCHYMA.

Comment

Immunohistochemical and special stains:

- Elastic (slides C4, C7) ---------- positive for visceral pleural invasion
- Mucicarmine (slide C4) ---------- negative
- CK 7 (slide C4) ---------- positive
- CK 20 (slide C4) ---------- negative
- TTF1 (slide C4) ---------- positive
- Napsin (slide C4) ---------- positive
- Pax 8 (slide C4) ---------- negative
- Thyroglobulin (slide C4) ---------- negative

This immunoprofile is compatible with a lung primary.

[redacted]

[redacted]

Synoptic Worksheet

C. Right lower lobe:

Specimen:	Lobe(s) of lung: Right lower
Procedure:	Lobectomy
Specimen Integrity:	Intact
Specimen Laterality:	Right
Tumor Site:	Lower lobe
Tumor Focality:	Unifocal
Histologic Type:	Adenocarcinoma, mixed subtype
Histologic Grade:	G1: Well differentiated
Tumor Size:	Greatest dimension: 4.0 cm Additional dimension: 4.0 cm Additional dimension: 2.0 cm
Visceral Pleura Invasion:	Present
Tumor Extension:	Not applicable

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Bronchial Margin:	Uninvolved by invasive carcinoma Involvement by squamous cell carcinoma in situ (CIS) not applicable
Vascular Margin:	Uninvolved by invasive carcinoma
Parenchymal Margin:	Not applicable
Parietal Pleural Margin:	Not applicable
Chest Wall Margin:	Not applicable
Other Attached Tissue Margin:	Not applicable
Treatment Effect:	Not applicable
Lymph-Vascular Invasion:	Present
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT2a: Tumor greater than 3 cm, but 5 cm or less in greatest dimension surrounded by lung or visceral pleura, without bronchoscopic evidence of invasion more proximal than the lobar bronchus (ie, not in the main bronchus); or Tumor 5 cm or less in greatest dimension with any of the following features of extent: involves main bronchus, 2 cm or more distal to the carina; invades the visceral pleura; associated with atelectasis or obstructive pneumonitis that extends to the hilar region but does not involve th the entire lung
Regional Lymph Nodes (pN):	pN0: No regional lymph node metastasis Nodes examined: 23 Nodes involved: 0
Distant Metastasis (pM):	Not applicable

Clinical History
Right VATS. Lobectomy.

Specimen(s) Received
A: #7 Lymph node
B: #11R lymph node
C: Right lower lobe
D: Posterior segment right upper lobe
E: 4R lymph node
F: Inferior portion of right middle lobe wedge

Gross Description

Specimen A is received in formalin labeled with the patient's name, medical record number and is labeled as "#7 lymph node." It consists of four lymph nodes that measure 0.7 to 1.6 cm in greatest dimension. The specimen is submitted in its entirety in cassette "A1." Dictated by [redacted]

Specimen B is received in formalin labeled with the patient's name, medical record number and is labeled as "11R lymph node." It consists of four lymph nodes that measure 0.5 to 0.6 cm in greatest dimension. The specimen is submitted in its entirety in

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cassette "B1." Dictated by

Specimen C is received fresh labeled with the patient's name, medical record number and is labeled "right lower lobe." It consists of a lobe of lung that measures 17.5 x 11.7 x 4.8 cm and weighs 229 grams. There is a firm, well-circumscribed mass that measures 4.0 x 4.0 x 2.0 cm. The mass abuts the bronchial margin and puckers the overlying pleura. The remainder of the lung is unremarkable. The bronchial margin is submitted for frozen section diagnosis and the contents of the cryoblock are transferred to cassette "C1FS." Sections are submitted as stated below. Dictated by

C2: Vascular margin
C3: One bronchial lymph node submitted whole
C4: Tumor and inked green pleura
C5: Tumor and bronchus
C6: Tumor and representative normal lung
C7: Representative tumor
C8: Representative normal lung

Specimen D is received in formalin, labeled with the patient's name, medical record number and designated "posterior segment of right upper lobe." The specimen consists of lung tissue measuring 6.1 x 2.3 x 1.2 cm. No lesions are identified within the lung parenchyma. The parenchymal margin is inked blue. Representative sections are submitted in cassettes "D1" and "D2." Dictated by

Specimen E is received in formalin, labeled with the patient's name, medical record number and designated "4R lymph node." The specimen consists of four pieces of soft tissue measuring 2.6 x 2.3 x 0.8 cm in aggregate. Multiple lymph nodes are identified and vary in size between 0.5 cm and 0.3 cm in greatest diameter. The specimen is submitted in its entirety as stated below. Dictated by

E1: At least three lymph node, whole
E2: At least four lymph nodes, whole

Specimen F is received in formalin, labeled with the patient's name, medical record number and designated "inferior portion of right middle lobe wedge." The specimen consists of lung tissue measuring 3.1 x 1.4 x 1.1 cm and is grossly unremarkable. Parenchymal margin is inked blue. The specimen is submitted entirely in cassette "F1." Dictated by

Intraoperative Consult Diagnosis

C1FS: RIGHT LOWER LOBE BRONCHIAL MARGIN (FROZEN SECTION): NEGATIVE FOR CARCINOMA.
Frozen section results were communicated to the surgical team and were repeated back by on out at

Pathologist:

Microscopic Description

Microscopic examination performed.

ANALYTE SPECIFIC REAGENT (ASR) DISCLAIMER:
Some of the above tests may use Class I ASRs. These tests were developed and their performance characteristics determined by They have not been cleared or approved by the Federal Drug Administration (FDA). The FDA

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does not require these tests to go through premarket FDA review. These tests are used for clinical purposes, and should not be regarded as investigational or for research. [redacted] laboratory is certified under the Clinical Laboratory Improvement Amendments (CLIA) as qualified to perform high complexity clinical laboratory testing.

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